6.18 Chronic Pancreatitis

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain never fully goes away." • Drinks alcohol every day **Objective** • Epigastric pain for a long time • Calcification seen on imaging	**ACTUAL** **Chronic pain** related to pancreatic damage as evidenced by persistent abdominal pain for months.	Pain at a level that lets him get on with his day.	• Work out a regular analgesic plan with the physician. • Help him stop drinking and refer him to services. • Teach pain measures that do not involve drugs. • Watch for dependence on analgesics.	Pain manageable: **goal met**.
Subjective • "My stool is greasy and floats." **Objective** • Fatty stools (steatorrhea) • Lost 5 kg	**ACTUAL** **Imbalanced nutrition: less than body requirements** related to malabsorption as evidenced by steatorrhea and weight loss.	Weight stays steady.	• Give pancreatic enzymes with every meal and every snack, as prescribed. • Suggest small meals, often. • Refer to the dietitian. • Weigh once a week.	Weight steady: **goal met**.
Subjective • *No specific complaint* **Objective** • Islet cells lost • Fasting glucose going up	**RISK** **Risk for unstable blood glucose level** as evidenced by risk factors: pancreatic endocrine insufficiency.	Blood glucose stays on target.	• Check blood glucose. • Teach the symptoms of high and low blood sugar. • If diabetes develops, refer to the diabetes educator.	Controlled: **goal met**.